

FREE FIRST CHAPTER

Your Body's Own GLP-1

A Woman's Guide to Weight, Hormones, and Appetite in the Ozempic Era

Rev. Dr. LaVeena B. Archers

This is the opening chapter, shared free. It is education and support, not medical advice, and it will never tell you to start or stop a medication. That belongs with you and your own prescriber.

PART I — THE HORMONE YOU ALREADY HAVE

Chapter 1 — The biggest health story of our time

A woman I will call Renata sat across from me last spring and said the thing I now hear almost every week. Everyone I know is on it, or thinking about it, or lying about it. She was fifty-one, tired in the particular way that midlife can make a woman tired, and she had come to me for a straight answer she could not seem to find anywhere else. Her sister had lost a great deal of weight on one of the new injections and looked, in Renata's words, like herself again. Her own doctor had offered her a prescription in the last four minutes of a fifteen-minute visit. A wellness account she followed swore the whole thing was poison, and that a cheap supplement did the same job. She wanted to know who was telling the truth, and what it would do to her body. Mostly she wanted one honest person to sit with her while she decided.

That conversation is the reason for this book.

The story everyone is living

Something large is happening, and we should name it before we go anywhere else. In 2025, about one in eight American adults told a national KFF survey they had used one of these medicines at some point, most often for diabetes or for weight, and the share using them specifically for weight had climbed quickly. Millions of those users are women. I have practiced in health and wellness for over thirty years, long enough to have watched trends rise and fade, and I can tell you this is not one of those. The speed and the scale have no real precedent for a medicine that changes how much a person wants to eat.

I feel the size of it in my own practice. A few years ago, almost no one crossed my path asking about weight-loss injections. Now some version of the question comes up nearly every week, from women who are already on one of these medicines, women whose sisters or friends are, women who tried to get one and could not afford it, and women who are frightened of them and want to know whether there is another way. No health fashion of years past ever filled the room like this.

You have felt it even if you have never held one of the pens. It is in the before-and-after photos, the comparisons whispered at school pickup, the sudden slimness of a public figure, the jokes on late-night television, the unspoken tension in your own family. The drugs go by their brand names, and you likely already know some of the story. A class of medicine first built for type 2 diabetes turned out to quell appetite so well that it has reshaped a national conversation about food, willpower, and the body in the space of a couple of years.

Two camps, and the empty middle

When something moves this fast, the loudest voices tend to sort into two camps, and that is exactly what has happened here.

On one side stand many good doctors, and behind them a pharmaceutical industry with vast resources. Their message runs roughly like this. Obesity is a chronic medical condition, these medicines treat it well, the shame was always misplaced, and you would be foolish to white-knuckle your way through a problem that finally has a real pharmaceutical answer. There is real truth in that, and it deserves its full weight. There is also a great deal of money riding on you believing that the answer is always a prescription.

On the other side stands the wellness internet, and its message is close to the opposite. The drugs are dangerous, unnatural, and a scam, your body can do all of this on its own, and here is a supplement, a tea, or a protocol that works just as well without the risk. There is real truth in that too, because a body that is well supported can do a great deal on its own. And there is money riding on this side just the same, in every powder and protocol sold with a promise it cannot keep. I hope to take what is real in each camp and set aside what is only being sold.

What troubles me about both camps is the same thing. Each is so intent on winning the argument that it forgets the actual woman standing in the middle of it, the one who wants to know what is true and what is right so she can decide for herself about her body. She reads the doctor who tells her the drug is the obvious answer, and she reads the influencer who tells her the drug is poison and the supplement is the secret, and she comes away more confused and more alone than when she began. I have watched that confusion up close, and I find it a kind of small cruelty.

What is missing, and what Renata could not find, is the middle ground. A place where the drugs are weighed fairly, as a real tool that has helped a great many people, with their costs and their unknowns laid out in plain sight. A place where the natural approaches are taken seriously for what they do, and held to the truth about what they cannot do. I wrote this book to stand in that middle, as steadily as I can, and to empower you to find what is right for your own body.

The one idea that changes everything

One idea reorganizes the whole conversation. Your body already makes GLP-1.

GLP-1 is a hormone. You have been producing it your whole life, in the lining of your own gut,

every time you eat. It is one of the signals your body uses to tell your brain that food has arrived, that you are growing full, that the meal can end. It helps steady your blood sugar after you eat, it slows the pace at which your stomach empties, and it turns down what many people call food noise, the constant background hum of wanting the next bite. The new medicines borrow this system rather than invent it. Most of them are built to resemble your own GLP-1 closely enough to lean on machinery your body was already running, and they hold that signal switched on far longer and far more strongly than your own hormone ever would.

Once you see the drugs this way, as a powerful amplifier of a system you already own, the whole map changes. The question stops being drugs against nature, as though those were two enemy countries. The real question becomes how you want to support this one system in your body, how much support you need, and at what cost, given your life and your history. Food supports it. Movement supports it. Sleep supports it. The condition of your gut supports it. And at the far end of that spectrum sit the medicines, which push the same system harder and faster than food and habits can. Each place along it does something the others cannot, and the whole art is knowing which one your body needs. It is one continuous story, from your breakfast to the pen in the refrigerator, and you get to decide where on that spectrum you belong.

What the drugs do, and I will be fair

First, the medicines deserve their due, and you deserve to know I am not against the world of medicine. The trial evidence behind them is real, and in places it is remarkable.

In the large trial that brought semaglutide to the world for weight, adults with obesity lost on average close to fifteen percent of their body weight over about sixteen months, while the group given a dummy injection lost only a little. For the newer medicine tirzepatide, the average losses in its landmark trial climbed higher, into the low twenties as a share of a person's body weight at the higher doses. Results like these had never been seen from a medicine you inject at home. They belong to a different order from the modest diet drugs that came before.

The benefits do not stop at the scale. In a large trial of adults who carried extra weight and already had heart disease, semaglutide lowered the risk of heart attacks, strokes, and cardiovascular death by roughly twenty percent, in people who did not have diabetes at all. That twenty percent is a relative figure, a fifth off a risk that was already fairly small for any one person, so the change in absolute numbers is more modest than it sounds. That is a serious finding about a serious outcome, and it is part of why no one gets to wave these drugs away as vanity in a syringe. For some women, this is a gift. I have watched it be one.

And the real costs, which I will not look away from

Now the other half of the truth, told just as fairly. These medicines carry real costs, and some of them fall hardest on exactly the women most likely to take them.

There is the matter of muscle. When weight comes off this fast, a meaningful share of what is lost can be lean tissue rather than fat, muscle included, by many estimates around a quarter of the total, and sometimes more, with the faster losses tending to cost the most. For a woman in midlife, who is already losing muscle and bone with each passing year, that is no small footnote. Muscle is what keeps you strong, steady on your feet, and metabolically healthy into your later decades.

Losing it carelessly is a price you can go on paying for years. It can be protected, and I will show you how, and why the protein and the strength work stop being optional the moment you pick up one of these pens.

There is the money, often hundreds of dollars a month, and the tangle of insurance around it. There is the open question of how long a person is meant to stay on a medicine like this, and what happens when you stop, because appetite tends to return and, for many people, so does some of the weight. There are the common side effects, the nausea and the gut complaints, and the rarer risks that any powerful medicine carries. And there is a whole shadow market of compounded and copycat versions, sold online with big promises and thin oversight, which I want you to understand clearly before you ever consider one.

None of this makes the drugs inherently bad, but it does make them a serious decision, the kind you make with your eyes open and with a prescriber who knows your history. My work is to help you see the whole picture before you choose, whichever way you choose, with the advice and assistance of your own practitioner.

ASK YOUR PRESCRIBER

If you are weighing one of these medicines, carry these questions into the room with you.

- How will we protect my muscle while the weight comes off?
- What will this cost me each month, and what will my insurance cover?
- How long am I meant to stay on it, and what happens if I stop?
- Which side effects should I expect, and which ones mean I should call you?

The trouble with nature's Ozempic

While the doctors and the drug companies were talking, the wellness market went looking for the natural version to sell you, and it settled on a few favorites. The most famous is a plant compound called berberine, crowned nature's Ozempic in a wave of viral videos.

The truth about it is the same truth every supplement here will get. Berberine is not nothing, and I have taken it myself and felt some benefit, though I have never taken any of the medicines. In a handful of studies, berberine has produced modest reductions in weight, on the order of a few pounds, along with some effect on blood sugar. That is worth knowing. It is also nowhere near the same category as the drugs it is compared to. The weight change is a fraction of what the medicines produce, the studies are small and mixed, and calling it nature's Ozempic is a marketing line rather than a medical fact. It is also not a casual add-on. Berberine can interact with prescription medicines, so clear it with your own prescriber before you start, and leave it alone if you are pregnant or nursing. You cannot make a good decision on a foundation of hype.

MYTH VS. REALITY

Myth: berberine is nature's Ozempic and does the same job as the drugs, without the risk.

Reality: in a handful of studies berberine produced modest weight loss, on the order of a few pounds, a fraction of what the medicines produce, from studies that are small and mixed. It can also interact with prescription medicines, so clear it with your prescriber first.

Why women, why now

There is a reason so much of this story runs through the lives of women in their middle years, and it has less to do with vanity than with biology and timing.

We women are driving this boom, and we are doing it in the season of life when the body rewrites its own rules. As estrogen shifts through the years around menopause, weight tends to settle differently, especially around the middle, and the old habits that used to hold it steady stop working through no fault of your own. Muscle and bone grow harder to keep and easier to lose. Sleep frays, stress runs high, and the appetite signals turn noisier. This is precisely the terrain where these medicines are most tempting, and precisely where their cost to muscle, and their still-unsettled effect on bone, matters most. A book about GLP-1 that ignored the female body would be missing the very center of the story. This one puts you there.

What this book is, and what it is not

So this is my promise to you, and what comes next.

I am going to explain the system your body already runs, in clear and simple language, so that you understand appetite and blood sugar from the inside out. Then we will walk the whole spectrum together, from the food and the habits that support your own GLP-1, through the gut and the muscle and the sleep, all the way to the medicines themselves, weighed fairly and squarely. I will give you an easier way to work out whether a drug belongs in your life, real guidance for doing it well if you choose it, and a real plan for the foundations whether you use one or not.

Wherever you are right now, you belong here, reading these pages. Maybe you are already on one of these medicines and want to do it wisely. Maybe you are weighing it and cannot get a straight answer. Maybe you have decided it is not for you and want to do everything you can on your own. Maybe you are coming off one and are frightened the weight will find you again. All four of those women are here, and this book is written for each of them, and for you.

This book is not a prescription, and it is not medical advice for your particular body. I hold a doctorate in natural medicine and board certifications in holistic functional medicine and functional nutrition, and I am not a conventional medical doctor, so I do not practice medicine. As a lifelong researcher and teacher of wellness, my work is to educate and support you, never to diagnose you, treat you, or tell you to start or stop any medication. Every drug decision in these pages belongs with you and the prescriber who knows your history. What I can do is hand you the honest science and the whole map, so that the choice you make is yours and fully informed.

Renata took several months to decide. She read, she asked hard questions, she worked on her foundations first and felt some things improve. In the end she chose to try one of the medicines, with a trusted doctor, while keeping every habit we had built together. That was the right choice

for her body. Yours may be different, and it will be no less right for being different.

One more thing before we begin in earnest. You do not need to read with a highlighter and a sense of dread, hunting for a single right answer. There is no single right answer, only the right way for your body, at this point in your life, given everything you know and everything you value. Read the way you would sit with a trusted friend who happens to research the science, at your own pace, taking what serves you and setting aside what does not yet apply. Carry that spirit with you as we turn, now, to the hormone itself.

THE BOTTOM LINE

Your body already makes GLP-1, and these medicines borrow that system rather than invent it. So the question stops being drugs against nature and becomes how much support your own system needs, and at what cost to you. My work here is to hand you the whole map so the choice stays yours, made with the prescriber who knows your history.

Keep reading.

That was Chapter 1 of twenty-one. The rest of the book walks through the drugs and the real trial data, what they do well and what they cost, the muscle-and-bone problem women can least afford, and every evidence-based way to support the GLP-1 your own body already makes.

The full book is on Amazon Kindle.

Find it, and the free GLP-1 Support Kit, at laveenaarchers.com

Rev. Dr. LaVeena B. Archers is a Doctor of Natural Medicine and board-certified holistic practitioner. She is not a conventional medical doctor. Her role is to educate and support, not to diagnose, treat, or prescribe.